THE WHOLE TREATMENT JOURNEY

Tucatinib + capecitabine + trastuzumab: the whole pathway

Advanced / metastatic · previously treated HER2-positive breast cancer

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; IV trastuzumab every 21 days. Continue while beneficial and tolerated, with clinician-approved changes.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Days 1–14	Days 15–21	Next cycle	Disease review
Cancer medicines NCCP 00936	Confirm prior HER2 therapies and DPD result	Tucatinib + capecitabine; trastuzumab Day 1	Tucatinib continues; capecitabine break	Review before any restart / next infusion	Continue, modify or change treatment according to benefit / toxicity
Surgery EARLY	No routine curative surgery sequence from this regimen	Local procedures only if indicated	Coordinate interruptions with MDT	Review symptoms and functional needs	Site-specific / CNS MDT when needed
Radiotherapy EARLY	Assess need for symptom or brain-directed treatment	Any concurrent RT needs an explicit plan	Do not assume capecitabine can continue with RT	Coordinate holds / restart with radiation team	Imaging and local treatment depend on disease site
Blood tests NCCP 00936	FBC, renal, liver; DPD testing before capecitabine	Extra tests if symptoms / toxicity	Pre-cycle bloods and symptom review	FBC / renal per cycle; liver at least every 3 weeks	Check dehydration, hepatic toxicity and interactions
Heart checks NCCP 00936 · ESC	Baseline echo / LVEF	Report chest pain or breathlessness	Symptoms need assessment between visits	Repeat echo according to HER2 plan, commonly 12-weekly	Risk / abnormalities may shorten interval
Cancer imaging METASTATIC · MARKERS	Baseline disease sites; brain assessment as indicated	No scan required simply because Day 14 is reached	Imaging schedule independent of tablet break	Usually review response about every 2–4 months; individualise	Use consistent modalities; earlier imaging if concern

What each check is for

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Blood tests and treatment review

FBC, renal, liver; DPD testing before capecitabine → Extra tests if symptoms / toxicity → Pre-cycle bloods and symptom review → FBC / renal per cycle; liver at least every 3 weeks → Check dehydration, hepatic toxicity and interactions

Results may change whether a dose is given or held.

Ask / act: Confirm the booking and who reviews the result before treatment.

Sources: NCCP 00936

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Heart tests

Baseline echo / LVEF → Report chest pain or breathlessness → Symptoms need assessment between visits → Repeat echo according to HER2 plan, commonly 12-weekly → Risk / abnormalities may shorten interval

ECG measures rhythm; echo measures pumping function. They are different checks.

Ask / act: Keep the planned appointments and report symptoms between them.

Sources: NCCP 00936 · ESC

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Imaging and markers

Baseline disease sites; brain assessment as indicated → No scan required simply because Day 14 is reached → Imaging schedule independent of tablet break → Usually review response about every 2–4 months; individualise → Use consistent modalities; earlier imaging if concern

The purpose and frequency depend on early versus advanced disease.

Ask / act: Ask which test is next and what decision its result will inform.

Sources: METASTATIC · MARKERS

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Bone health with endocrine treatment

At endocrine / ovarian-suppression planning; repeat by risk

Low oestrogen can affect bone strength.

Ask / act: Ask whether DEXA, fracture-risk assessment or bone protection is appropriate.

Sources: BONE · NOGG

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Who arranges and reviews each check?

At initiation and every handover

A recommended test can be missed if responsibility is unclear.

Ask / act: Ask the team to document the booking owner, result reviewer, due window and escalation route.

Sources: NCCP 00936

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-17 · Draft — independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.83.8